Active Home Healthcare LLC

OASIS

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Las Vegas NV 89146
Phone: (775)471-9815 Fax: (702)726-9551
NPI: 1548015142

STAFF INFO
Name: Dizon, Bright RN
Patient INFO
Name: CARRIERE, EDWARD
DOB: 1948-04-04

OASIS Assessment: OASIS E2 - ST START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number:
297370

(M0014) Branch State:
Branch State: **Nevada**

(M0016) Branch ID Number:

(M0018) Attending Physician (NPI):
1073568903

Physician:
HILLOCK, RONALD

Physician Address:
7455 W WASHINGTON AVE #160 LAS VEGAS NV 89128

☐ Not Applicable

(M0020) Patient ID number:
116283

(M0030) Start of Care Date:
04/08/2026

(M0032) Resumption of Care Date:
mm/dd/yyyy

☑ Not Applicable

(M0040) Patient Name:

First Name:	Middle Name:	Last Name:	Suffix:
EDWARD		CARRIERE	

(M0066) Birth Date:
04/04/1948

(A0810) Sex:
◉ 1. Male ○ 2. Female

(M0050) Patient State of Residence:
State of Residence: **Nevada**

(M0060) Patient Zip Code:
89128

(M0063) Medicare Number:
3DM9R71CM27

☐ Not Applicable

(M0064) Social Security Number:

☑ Not Applicable

(M0065) Medicaid Number:

☑ Not Applicable

(M0080) Discipline Completing Assessment:
◉ 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Dizon, Bright RN

(M0090) Date Assessment Completed:
04/08/2026

(M0100) Assessment Reason:
◉ 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply:

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language:

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
◉ 0. No ○ 1.Yes ○ 9. Unable to determine

(B1300) Health Literacy:
How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?
○ 0. Never ◉ 1. Rarely ○ 2. Sometimes ○ 3. Often ○ 4. Always ○ 7. Patient declines to respond ○ 8. Patient unable to respond

(M0102) Date of Physician-ordered Start of Care (Resumption of Care):
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1255, Transportation, if date entered:
04/08/2026 ☐ NA - No specific SOC/ROC date ordered by physician

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
mm/dd/yyyy

(M0150) Current Payment Sources for Home Care:
(Mark all that apply)

- ☐ 0. None; no charge for current services
- ☑ 1. Medicare (Traditional fee-for-service)
- ☐ 2. Medicare (HMO/managed care/Advantage plan)
- ☐ 3. Medicaid (Traditional fee-for-service)
- ☐ 6. Title programs (for example, Title III, V, or XX)
- ☐ 7. Other government (for example, TriCare, VA)
- ☐ 8. Private insurance
- ☐ 9. Private HMO/managed care

- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1255) Transportation (NACHC©):

In the past 12 months, has lack of reliable transportation kept you from medical appointments, meetings, work or from getting things needed for daily living?

- (●) 0. Yes
- ( ) 1. No
- ( ) 7. Patient declines to respond
- ( ) 8. Patient unable to respond

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(M1000) Inpatient Facilities that patient was discharged within the past 14 days?

(Mark all that apply)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [x] NA - Patient was not discharged from an inpatient facility _[Go to M1021]

(M1005) Inpatient Discharge Date (Most Recent):

mm/dd/yyyy [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity (POC Element):

The patient is considered homebound because they have a symptomatic pathologic fracture of the right fibula with an expansile, infiltrative destructive lesion and soft-tissue extension on MRI, producing moderate tenderness and aching with weightbearing (pain 7/10) that limits safe ambulation and makes exiting the home without assistance unsafe. Objective findings supporting homebound status include: pathologic fracture of the right fibula on plain radiograph with bridging callus, an expansile destructive fibular lesion with soft-tissue extension on MRI concerning for sarcoma, a palpable mass with localized moderate tenderness on exam, and documented pain with weightbearing. These conditions create a risk of further fracture, acute pain exacerbation, and soft-tissue compromise with ambulation; the patient is elderly (77 years) and has comorbid chronic lymphocytic leukemia and is taking antiplatelet therapy (clopidogrel), which increase peri-procedural risk and complicate safe transport and unsupervised activity outside the home. Leaving the home requires a considerable and taxing effort because any weightbearing provokes significant pain, there is structural instability of the fibula due to the pathologic fracture and destructive lesion, and there is no home assistance available to provide safe transfer or supervised mobility. Allowable, limited absences are restricted to medically necessary trips (preoperative evaluation, diagnostic imaging, and planned surgical care) only. Given the need for pre-operative evaluation and coordination, peri-procedural risk management, skilled wound and pain management planning, and close monitoring of a malignant/pathologic fracture, these objective limitations meet Medicare homebound criteria and justify the medical necessity for skilled home health services.

Homebound Reason (POC Element):

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [x] Cane
  - [ ] Manual Wheel Chair
  - [x] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [x] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadprilegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [x] Leaving home is exhausting, extended rest periods to recover
- [ ] Extended respiratory recovery after exertion
- [x] Requires human assist due to unsteady gait/poor balance
- [x] Weakness and pain from recent surgery
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [x] Requires assistive device to ambulate

- [x] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis (POC Element (§484.60 (2.v))):

○ 1-Poor ○ 2-Guarded ● 3-Fair ○ 4-Good ○ 5-Excellent

Functional Limitations (POC Element (§484.60 (2.vi))):

○ 1 - Amputation
○ 2 - Bowel/Bladder (Incontinence)
○ 3 - Contracture
● B- Other (Specify)
s/p right fibular surgery
○ 4 - Hearing
○ 5 - Paralysis
● 6 - Endurance
● 7 - Ambulation
○ 8 - Speech
○ 9 - Legally blind
○ A - Dyspnea with minimal exertion

Emergency Preparedness (POC Element):

Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed patient specific emergency planning options
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Obtained the patient's emergency contact numbers for the medical record
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.
List of recommended items to have prepared/ready and available in the event of an emergency

Acuity Level:
Acuity Level: **Medium Risk or Medium Priority**

Emergency Triage:
Emergency Triage: **Category 2 - Not life threatening but would suffer severe adverse effect**

Additional Emergency Preparedness Information:

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives (POC Element (§484.60 (2.xv))):

Patient has Advance Directives? ● Yes ○ No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [ ] Power of Attorney
- [x] Living Will — Copies to Follow
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [ ] Full Code
- [ ] The patient has not executed any advance directives to health care.
- [x] The patient was provided with written and verbal information on advance directives.

Comments:

Has surrogate? ○ Yes ● No

Surrogate Name:

Surrogate Phone:

Health Screening:

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ○ Yes ● No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control (POC Element (§484.60 (2.i))):

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

PRIMARY DIAGNOSIS
C40.21 - Malignant neoplasm of long bones of right lower limb

Onset Date 04/08/2026

Severity
0 | 1 | 2 | (3) | 4 | ^

Onset | (Exacerbate)

OTHER DIAGNOSIS - 1
M84.469D - Path fx, unsp tibia and fibula, subs for fx w routn heal

Onset Date 04/08/2026

Severity
0 | 1 | 2 | (3) | 4 | ^

Onset | (Exacerbate)

OTHER DIAGNOSIS - 2
C91.10 - Chronic lymphocytic leuk of B-cell type not achieve remis

Onset Date 04/08/2026

Severity
0 | 1 | 2 | (3) | 4 | ^

Onset | (Exacerbate)

OTHER DIAGNOSIS - 3
I10. - Essential (primary) hypertension

Onset Date 04/08/2026

Severity
0 | 1 | (2) | 3 | 4 | ^

(Onset) | Exacerbate

OTHER DIAGNOSIS - 4
E78.5 - Hyperlipidemia, unspecified

Onset Date 04/08/2026

Severity
0 | 1 | (2) | 3 | 4 | ^

(Onset) | Exacerbate

(M1028) Active Diagnoses:
Comorbidities and Co-existing Conditions *– Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.*

- [ ] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [ ] **2. Diabetes Mellitus (DM)**
- [x] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up:
A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
68

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
186

BMI:
28.28

Comments:

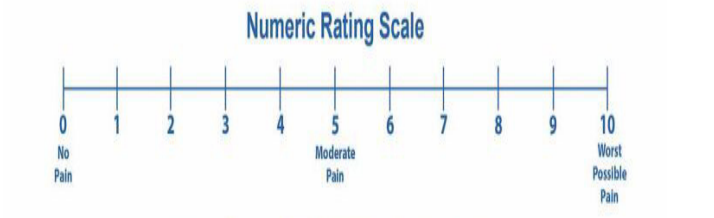
Vital Signs

Temperature: 97.8 Temporal
Respiratory (per minute): 18
Blood Pressure mm/Hg (R): /
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 98
Comments:

Pulse (per minute): Apical = Radial = 76
Weight:
Blood Pressure mm/Hg (L): 115 / 68

O2 Sat in: Room Air

Pain Assessment



4

Collected Using: [ ] Face Scale [x] Numeric Rating Scale

Pain Location and Description:

1 Location	Start Date	Activity	What makes pain worse?

Right lower extremity | 04/08/2026 | With Activity× | Movement×

Intensity 4×

Duration hours

What makes pain better? Med/Rest×

Does pain affect quality of life? Yes

Unable to Assess ☐

Description Throbbing×

Frequency Frequent×

Unable to Assess ☐

Type of Intervention Opiod Analgesic×

How well it is working Fairly Well×

Status Treat×

Comments:

(J0510) Pain Effect on Sleep:

Ask patient: *"Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"*

- ○ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ○ **1. Rarely or not at all**
- ● **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities:

Ask patient: *"Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"*

- ○ **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ○ **1. Rarely or not at all**
- ● **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities:

Ask patient: *"Over the past 5 days, how often you have limited your day to day activities (excluding rehabilitation therapy session) because of pain?"*

○ **1. Rarely or not at all** ○ **2. Occasionally** ● **3. Frequently** ○ **4. Almost Constantly** ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

Medication (POC Element (§484.60 (2.x))):

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	04/08/2026	Aspirin (Oral Pill)	81 mg 1 Tab Once a day	By mouth	NON-OPIOID ANALGESICS /	New
☐	04/08/2026	Clopidogrel (Oral Pill)	75 mg 1 Tab Once a day	By mouth	PLATELET AGGREGATION INHIBITORS /	New
☐	04/08/2026	Losartan (Oral Pill)	100 mg 1 Tab Once a day	By mouth	ANGIOTENSIN II INHIBITOR /	New
☐	04/08/2026	Gabapentin (Oral Pill)	400 mg Cap Three times a day	By mouth	ANTICONVULSANTS /	New
☐	04/08/2026	Rosuvastatin (Oral Pill)	20 mg 1 Tab Once a day	By mouth	ANTILIPEMIC AGENTS /	New
☐	04/08/2026	Amlodipine	10 mg 1 Tab Once a day	By mouth	CALCIUM CHANNEL BLOCKERS /	New
☐	04/08/2026	Metoprolol (Oral Pill)	100 mg 1 Tab Once a day	By mouth	BETA BLOCKERS/RELATED /	New
☐	04/08/2026	Acetaminophen/HYDROcodone (Oral Pill)	325-5 mg 1 Tab Once a day	By mouth	OPIOID ANALGESICS /	New

Medication Remarks

Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Allergies (POC Element):

Name	Reaction	Start Date

☐ **Add Allergies**

None known

(M2001) Drug Regimen Review:

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review _[Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ○ **1. Yes – Issues found during review**

- ◯ Not assessed/no information
- ◯ 9. NA – Patient is not taking any medications _[Go to O0110, Special Treatments, Procedures, and Programs]

(M2003) Medication Follow-up:

Did the agency contact a physician (or physician designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

◯ 0. No ◯ 1. Yes ◯ Not assessed/no information

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking	2. Indication Noted
	1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	☐
E. Anticoagulant	☐	☐
F. Antibiotic	☐	☐
H. Opioid	☑	☑
I. Antiplatelet	☑	☑
J. Hypoglycemic (including insulin)	☐	☐
Z. None of the Above	☐	

(M2010) Patient/Caregiver High-Risk Drug Education:

Has the patient/caregiver received instruction on special precautions for all high risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ◯ 0. No
- ● 1. Yes
- ◯ NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications

(M2020) Management of Oral Medications:

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. ***(NOTE: This refers to ability, not compliance or willingness.)***

- ◯ 0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.
- ● 1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.
- ◯ 2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.
- ◯ 3. Unable to take medication unless administered by another person
- ◯ NA - No oral medications prescribed

(M2030) Management of Injectable Medications:

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ◯ 0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times
- ◯ 1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.
- ◯ 2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection
- ◯ 3. Unable to take injectable medication unless administered by another person
- ● NA - No injectable medications prescribed.

(O0110) Special Treatments, Procedures, and Programs:

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐

H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations:

Pneumonia: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

Flu: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

Tetanus: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

TB: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

TB Exposure: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

Hepatitis B: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

COVID-19: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

Additional Immunization: ☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy

Comments:

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Neuromuscular:
☐ **No Problems Identified**

Level of Consciousness:
☑ **Awake** ☑ **Alert** ☐ **Lethargic** ☐ **Minimally Responsive** ☐ **Unresponsive** ☐ **Coma**

Impairment:
☐ **Speech** ☐ **Hearing** ☐ **Visual** ☐ **Tremors** ☐ **Numbness** ☐ **Tingling** ☐ **Vertigo** ☐ **Ataxia** ☐ **Decreased Sensitivity**

Speech Impairment (Specify):

Functional Assessment Staging Tool

☐ **N/A** ☐ **Fast Scale**

Psychosocial (POC Element (§484.60 (2.ii))):
☐ **Document as Clinical Narrative**

Educational level

Primary Language

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language
○ **Needs interpreter**

Learning
○ **Mental**
● **Physical**
○ **Psychosocial**
○ **Functional**

Emotions Reported:

○ **Angry**
○ **Discouraged**
○ **Withdrawn**

○ **Flat affect**
○ **Difficulty coping**
○ **Disorganized**

○ **Depressed** ○ **Recent** ○ **Long Term**

Treatment:

Evidence of abuse/neglect/exploitation:

○ **Potential**
○ **Verbal/Emotional**
○ **Physical**

○ **Actual**
○ **Financial**
● **No Evidence of Abuse/Neglect/Exploitation**

Unable to cope with altered health status, patient:

○ **Lacks motivation** ○ **Has unrealistic expectations** ○ **Denies problems** ● **Unable to recognize problems**

Sleep/Rest

● **Adequate** ○ **Inadequate**

No. of hours slept per night:
7

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☐Yes ☐No ☐NA

Did the patient have a job before admission? ○Yes ◉No

If yes, do they want to drive post-discharge? ☐Yes ☐No

If yes, do they want to return to work post-discharge? ○Yes ○No

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ○ **No (patient is rarely/never understood)** ● **Yes** ○ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ○ **None** ○ **One** ○ **Two** ● **Three** ○ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year
- ○ **0. Missed by > 5 years or no answer**
- ○ **1. Missed by 2-5 years**
- ○ **2. Missed by 1 year**
- ● **3. Correct**
- ○ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month
- ○ **0. Missed by > 1 month or no answer**
- ○ **1. Missed by 6 days to 1 month**
- ● **2. Accurate within 5 days**
- ○ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week
- ○ **0. Incorrect or no answer**
- ● **1. Correct**
- ○ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"
- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("something to wear")**
- ● **2. Yes, no cue required**
- ○ **Not assessed/no information**

B) Able to recall "blue"
- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("a color")**
- ● **2. Yes, no cue required**
- ○ **Not assessed/no information**

C) Able to recall "bed"
- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("a piece of furniture")**
- ● **2. Yes, no cue required**
- ○ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **15**

(C1310) Signs and Symptoms of Delirium (from CAM©):

A. Acute Onset of Mental Status Change:

Is there evidence of an acute change in mental status from the patient's baseline?

● **0. No** ○ **1. Yes**

B. Inattention:

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ● **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

C. Disorganized thinking:

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ● **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

D. Altered level of consciousness:

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- ***vigilant*** *– startled easily to any sound or touch* ☐ *lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch*
- ***stuporous*** *– very difficult to arouse and keep aroused for the interview*
- ***comatose*** *– could not be arouse*

- ● **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

(D0150) Patient Mood Interview (PHQ-2 to 9):

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	**No** ×	**Never or 1 day**
B. Feeling down, depressed, or hopeless	**No** ×	**Never or 1 day**
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^

	Presence	Frequency
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation:

How often do you feel lonely or isolated from those around you?

- ○ 0. Never
- ● 1. Rarely
- ○ 2. Sometimes
- ○ 3. Often
- ○ 4. Always
- ○ 7. Patient declines to respond
- ○ 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed):

(Mark all that apply)

- ☐ 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- ☐ 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- ☐ 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- ☐ 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- ☐ 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- ☐ 6. Delusional, hallucinatory, or paranoid behavior
- ☑ 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- ● 0. Never
- ○ 1. Less than once a month
- ○ 2. Once a month
- ○ 3. Several times each month
- ○ 4. Several times a week
- ○ 5. At least daily

(M1700) Cognitive Functioning:

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- ● 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- ○ 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
- ○ 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
- ○ 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- ○ 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- ● 0. Never
- ○ 1. In new or complex situations only
- ○ 2. On awakening or at night only
- ○ 3. During the day and evening, but not constantly
- ○ 4. Constantly
- ○ 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- ● 0. None of the time
- ○ 1. Less often than daily
- ○ 2. Daily, but not constantly
- ○ 3. All of the time
- ○ NA - Patient nonresponsive

Mental Status (POC Element (§484.60 (2.ii)):

- ● 1 - Oriented
- ○ 2 - Comatose
- ○ 3 - Forgetful
- ○ 4 - Depressed
- ○ 5 - Disoriented
- ○ 6 - Lethargic
- ○ 7 - Agitated
- ● 8 - Alert
- ○ 9 - Confused
- ○ 10 - Dementia
- ○ 11 - Assaultive
- ○ 12 - Delirious
- ○ 13 - Wanders
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert and oriented, patient dress is clean and appropriate cooperative with assessment, express feeling depressed but denies suicidal/homicidal ideation.

Has there been a sudden/acute change in their mental status? ☐ Yes ☐ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

Cultural:

Is religion important to the patient? ○ Yes ○ No

Patient's primary source of emotional support:

Eyes and Ears

(B1000) Vision:

Ability to see in adequate light *(with glasses or other visual appliances)*

- ● 0. Adequate – sees fine detail, such as regular print in newspapers/books
- ○ 1. Impaired – sees large print, but not regular print in newspapers/books
- ○ 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- ○ 3. Highly impaired – object identification in question, but eyes appear to follow objects
- ○ 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision Assessment:

- ☑ No Problems Identified
- ☐ Glasses
- ☐ Contact Lenses: ☐ Right ☐ Left
- ☐ Glaucoma: ☐ Right ☐ Left
- ☐ Blurred Vision: ☐ Right ☐ Left
- ☐ Macular Degeneration: ☐ Right ☐ Left
- ☐ Blindness: ☐ Right ☐ Left
- ☐ Cataracts: ☐ Right ☐ Left
- ☐ Other (Specify):

Comments:

(B0200) Hearing:

Ability to hear *(with hearing aid or hearing appliances if normally used)*

- [x] 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- [ ] 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- [ ] 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- [ ] 3. Highly impaired – absence of useful hearing

Hearing Assessment:

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Hearing Aids:
- [ ] Right
- [ ] Left

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Chest/Thorax

Chest Pain:

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Nausea
- [ ] Other (Specify):

Associated with:

- [x] Shortness of Breath
- [x] Activity
- [ ] Rest
- [ ] Diaphoresis
- [ ] Frequency/Duration:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- [ ] 0. Patient is not short of breath
- [x] 1. When walking more than 20 feet, climbing stairs
- [ ] 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- [ ] 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- [ ] 4. At rest (during day or night)

Respiratory Status:

Respiratory Distress Indicators:

- [ ] Dyspnea:
- [ ] At Rest
- [ ] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor
- [ ] O2 Liters per minute:
- [ ] O2 Saturation (%):

Frequency/Timing of Oxygen Use:

- [ ] PRN
- [ ] Continuous
- [ ] Only at bedtime
- [ ] Other (Specify):

Route/Delivery:

- [ ] Nasal Cannula
- [ ] Trach Cuff
- [ ] Mask (Specify):
- [ ] Other (Specify):

GASTROINTESTINAL & GENITOURINARY

(M1870) Feeding or Eating:

*Current ability to feed self meals and snacks safely. Note: This refers only to the process of **eating**, **chewing**, and **swallowing**, not preparing the food to be eaten.*

- [ ] 0. Able to independently feed self.
- [x] 1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- [ ] 2. Unable to feed self and must be assisted or supervised throughout the meal/snack.
- [ ] 3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- [ ] 4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- [ ] 5. Unable to take in nutrients orally or by tube feeding.

Appetite (%): 80%

Appetite Percentage of (Meals per day): 3

Snacks (Per day):

Appetite:

- [ ] Increase
- [ ] Decreased
- [x] Unchanged

Intake:

- [x] Adequate
- [ ] Inadequate

Nutritional Abnormalities:

- [ ] Anoxeria
- [ ] Cachexia
- [ ] Decreased Appetite
- [ ] Constipation

(K0520) Nutritional Approaches:

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	☐
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	☐
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	☐
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	☑
Z. None of the above	☐

#16 Nutritional Requirements (POC Element (§484.60 (2.ix))):

- [ ] NPO
- [ ] No Added Salt
- [x] Low Salt
- [x] Low Fat
- [x] Low Cholesterol
- [ ] No Concentrated Sweets
- [ ] Controlled Carbohydrates
- [ ] Regular
- [ ] Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
◯ Without reason, has lost more than 10 lbs in the last 3 months	15	◯ Does not always have enough money to buy foods need	10
		Total	15

Nutritional Health Screen	Score	Nutritional Health Screen	Score
● Has an illness or condition that made pt change the type and/or amount of food eaten	10	○ Eats alone most of the time	5
○ Has open decubitus, ulcer, burn or wound	10		
○ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
○ Has a tooth/mouth problem that makes it hard to eat	10	○ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
○ Has 3 or more drinks of beer, liquor or wine almost every day	10	○ Frequently has a diarrhea or constipation	5
		Total	15

● Good Nutritional Status (Score 0 - 25) ○ Moderate Nutritional Status (Score 25 - 54) ○ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:
SN educated the patient on the prescribed diet and the importance of proper nutrition and hydration.

Aspiration Risk Assessment

☐ EAT-10 Assessment Tool ☐ Yale Swallow Protocol ☐ Not Applicable

(M1620) Bowel Incontinence Frequency:

● 0. Very rarely or never has bowel incontinence
○ 1. Less than once weekly
○ 2. One to three times weekly
○ 3. Four to six times weekly
○ 4. On a daily basis
○ 5. More often than once daily
○ NA - Patient has ostomy for bowel elimination
○ UK - Unknown

(M1630) Ostomy for Bowel Elimination:

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

● 0. Patient does not have an ostomy for bowel elimination
○ 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
○ 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

● 0. No ○ 1. Yes ○ NA - Patient on prophylactic treatment ○ UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

○ Patient is incontinent
- ○ Day time
- ○ Night time
- ○ Day and Night
- ○ Timed-voiding defers incontinence
- ○ Occasional stress incontinence

● No incontinence or catheter (includes anuria or ostomy for urinary drainage)

○ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

● 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
○ 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage:

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persis tent blue or purple hues.

Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

● 0. No **[GO TO M1340]**
○ 1. Yes, patient has BOTH observable and unobservable stasis ulcers
○ 2. Yes, patient has observable stasis ulcers ONLY
○ 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non┐removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

○ 1. One ○ 2. Two ○ 3. Three ○ 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable:

○ 1. Fully granulating ○ 2. Early/partial granulation ○ 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- ◯ 0. No _[GO TO N0415]
- ◯ 1. Yes, patient has at least one observable surgical wound
- ● 2. Surgical wound known but not observable due to non-removable dressing/device _[GO TO N0415]

(M1342) Status of Most Problematic Surgical Wound that is Observable:

◯ 0. Newly epithelialized ◯ 1. Fully granulating ◯ 2. Early/partial granulation ◯ 3. Not healing

Skin Color:

☑ Normal ☐ Pale ☐ Jaundice ☐ Cyanotic

Skin Turgor:

☐ Good ☑ Fair ☐ Poor

Skin Abnormalities:

☑ Dry ☑ Warm ☐ Cool ☐ Rash ☐ Ulcer ☐ Ostomy ☑ Incision ☐ Diaphoretic ☐ Other (Specify):

Comments:

Norton Scale for Pressure Ulcer Risk Assessment:

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	◯ Good	● Fair	◯ Poor	◯ Very Bad
Mental Condition	● Alert	◯ Apathetic	◯ Confused	◯ Stuporous
Activity	◯ Ambulant	● Walks With Help	◯ Chairbound	◯ Bedfast
Mobility	◯ Full	● Slightly Impaired	◯ Very Limited	◯ Immobile
Incontinence	● None	◯ Occassional	◯ Usually Urinary	◯ Urinary and Fecal
			Total Score:	17

Wound

Wound Worksheet:

Fetch Wounds From Previous Wound Note

Wound 1

Location: Right lower leg	Size: 0 x 0 x 0	Pressure Injury Stage: -
Type: Surgical	Drainage: -	Amount Of Drainage: -
Odor: -	Color: -	Tunneling/Undermining: -
Tunneling Location: -	Tunneling Length (cm): -	Undermining Location: -
Undermining Length (cm): -	Surrounding Tissue: -	Wound Base: -
Level Of Pain: -	Edema: -	Stoma: -
Slough and/or Echar: -	Slough (%): -	Eschar (%): -
Granulation (%): -	Pink/Red (%): -	Necrotic (%): -
Wound Status: -	Wound Description: -	Satisfaction of Return Demonstration: - -
Patient's Response to Wound Care: - - undefined	Wound Care: Surgical wound known but not observable due to non-removable dressing/device	
Onset Date: 2026-04-08		

SAFETY & RISK ASSESSMENT

Home Environment/Home Safety Assessment

☐ Architectural Barriers	☐ No fire extinguisher	☐ No emergency exit plan
☑ Medications are improperly stored	☐ Inadequate electricity/cooling/heating	☐ Rodent/insect infestation
☐ No telephone	☐ Inadequate supply of water	☐ Unsafe electrical wiring
☑ Cluttered surroundings	☐ Inadequate sanitation/plumbing	☐ Nonworking stove/refrigerator
☐ Inadequate railing	☐ Inadequate smoke detectors on all levels of home	☐ Exposed flammables
☐ Unsafe oxygen use	☐ Unsafe storage of equipments	☐ Inadequate Lighting

Others (Specify):

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL (Bathing, dressing, toileting, eating/feeding, bowel):

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances):

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Psychosocial Support:

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Assistance with doctors appointment, delivery of medications:

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Management of Finances:

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Comments:

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	○ Patient Lives alone around the clock	○ Patient Lives alone Regular daytime	○ Patient Lives alone Regular Night time	● Patient Lives alone Short-term Assistance	○ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	○ Patient lives with other person(s) in the home Around the clock	○ Patient lives with other person(s) in the home Regular daytime	○ Patient lives with other person(s) in the home Regular Night time	○ Patient lives with other person(s) in the home Short-term Assistance	○ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	○ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	○ Regular daytime	○ Patient lives in congregate situation Regular Night time	○ Patient lives in congregate situation Short-term Assistance	○ Patient lives in congregate situation No Assistance available

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety *(for example, due to cognitive impairment)*

- ○ 0. No assistance needed – patient is independent or does not have needs in this area
- ● 1. Non-agency caregiver(s) currently provide assistance
- ○ 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- ○ 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ○ 4. Assistance needed, but no non-agency caregiver(s) available

Safety Measures (POC Element (§484.60 (2.xi))):

- ● 1 - Bleeding precautions
- ○ 2 - O_2 precautions
- ○ 3 - Seizure precautions
- ● 4 - Fall precautions
- ○ 5 - Aspiration precautions
- ○ 6 - Siderails up
- ○ 7 - Elevate head of bed
- ○ 8 - 24 hr. supervision
- ● 9- Clear Pathways up
- ○ 10 - Lock w/c with transfers
- ● 11 - Infection control measures
- ● 12 - Walker/cane
- ○ 13 - Sharps Precaution
- ● 14 - Other:

ER/911 protocol, COVID-19, and universal precautions

Hospitalization Risk Assessment

(M1033) Risk for Hospitalization:

Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- ☐ 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- ☐ 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- ☐ 3. Multiple hospitalizations (2 or more) in the past 6 months
- ☐ 4. Multiple emergency department visits (2 or more) in the past 6 months
- ☐ 5. Decline in mental, emotional, or behavioral status in the past 3 months
- ☑ 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- ☑ 7. Currently taking 5 or more medications
- ☐ 8. Currently reports exhaustion
- ☐ 9. Other risk(s) not listed in 1 - 8
- ☐ 10. None of the above

Additional Risk Information:

Fire Assessment for Patient with Oxygen

☑ Patient is not using oxygen

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities:

Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- ***3. Independent*** *– Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper*
- ***2. Needed Some Help*** *– Patient needed partial assistance from another person to complete activities*
- ***1. Dependent*** *– A helper completed the activities for the patient.*
- ***8. Unknown***
- ***9. Not Applicable***

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓	○	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓	○	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	✓	○	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓	○	○	○	○

(GG0110) Prior Device Use:

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury ***(Check all that apply)***

- [ ] A. Manual wheelchair
- [ ] B. Motorized wheelchair and/or scooter
- [ ] C. Mechanical lift
- [ ] D. Walker
- [ ] E. Orthotics/Prosthetics
- [x] Z. None of the above

(GG0130) Self Care

(GG0130) SOC/ROC Performance:

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance –
Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130A) A. Eating:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130B) B. Oral Hygiene:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130C) C. Toileting Hygiene:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130E) E. Shower/Bathe Self:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130F) F. Upper Body Dressing:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130G) G. Lower Body Dressing:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130H) H. Putting on/Taking Off Footwear:
- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(M1800) Grooming:

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)
- [ ] 0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- [x] 1. Grooming utensils must be placed within reach before able to complete grooming activities.
- [ ] 2. Someone must assist the patient to groom self.
- [ ] 3. Patient depends entirely upon someone else for grooming needs

(M1810) Ability to Dress Upper Body:

*Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:*
- [ ] 0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- [x] 1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- [ ] 2. Someone must help the patient put on upper body clothing.
- [ ] 3. Patient depends entirely upon another person to dress the upper body.

(M1820) Ability to Dress Lower Body:

*Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:*
- [ ] 0. Able to obtain, put on, and remove clothing and shoes without assistance.
- [x] 1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- [ ] 2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- [ ] 3. Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

*Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).***
- [ ] 0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- [x] 1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.

- ◯ 2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- ◯ 3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- ◯ 4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- ◯ 5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- ◯ 6. Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ◯ 0. Able to get to and from the toilet and transfer independently with or without a device
- ● 1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- ◯ 2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- ◯ 3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- ◯ 4. Is totally dependent in toileting.

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ◯ 0. Able to manage toileting hygiene and clothing management without assistance.
- ● 1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- ◯ 2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- ◯ 3. Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ◯ 0. Able to independently transfer.
- ● 1. Able to transfer with minimal human assistance or with use of an assistive device.
- ◯ 2. Able to bear weight and pivot during the transfer process but unable to transfer self.
- ◯ 3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- ◯ 4. Bedfast, unable to transfer but is able to turn and position self in bed.
- ◯ 5. Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion:

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ◯ 0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- ● 1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
- ◯ 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- ◯ 3. Able to walk only with the supervision or assistance of another person at all times.
- ◯ 4. Chairfast, unable to ambulate but is able to wheel self independently.
- ◯ 5. Chairfast, unable to ambulate and is unable to wheel self
- ◯ 6. Bedfast, unable to ambulate or be up in a chair.

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right:

The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170B) B. Sit to lying:

The ability to move from sitting on side of bed to lying flat on the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed:

The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170D) D. Sit to stand:

The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer:

The ability to transfer to and from a bed to a chair (or wheelchair).

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170F) F. Toilet transfer:

The ability to get on and off a toilet or commode.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 07. Patient refused
- ◯ 09. Not applicable

- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170G) G. Car transfer:

The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet:

Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns:

Once standing, the ability to walk 50 feet and make two turns.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet:

Once standing, the ability to walk at least 150 feet in a corridor or similar space.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces:

The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170M) M. 1 step (curb):

The ability to go up and down a curb and/or up and down one step. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170N) N. 4 steps:

The ability to go up and down four steps with or without a rail. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170O) O. 12 steps:

The ability to go up and down 12 steps with or without a rail.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170P) P. Picking up object:

The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

● 0. No _**[GOTO M1600]** ◯ 1. Yes ◯ (-) Not assessed/no information

(GG0170R) R. Wheel 50 feet with two turns:

Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ (-) Not assessed/no information

(GG0170S) S. Wheel 150 feet:

Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ (-) Not assessed/no information

DME and Supplies (POC Element (§484.60 (2.iii))):

DME Lookup

Durable Medical Equipment:
Cane, Walker, Wound Care Supplies

#18B Activities Permitted (POC Element (§484.60 (2.viii))):

- ◯ 1- Complete bedrest
- ◯ 2 - Bedrest/BRP
- ✓ 3 - Up as tolerated
- ✓ 4 - Transfer bed/chair
- ✓ 5 - Exercises prescribed
- ✓ 6 - Partial weight bearing
- ◯ 7 - Independent in home
- ◯ 8 - Crutches
- ✓ 9 - Cane
- ◯ A - Wheelchair
- ✓ B - Walker
- ◯ C - No restrictions
- ✓ D - Other (specify)

Assist with ADLs

Rehabilitation Potential (§484.60 (2.vi))):

- ◯ Poor
- ✓ Fair
- ◯ Good
- ◯ Excellent

Others

PLAN OF CARE ORDERS AND DISCHARGE PLAN

Plan of Care Orders

(POC Element):

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 2/wk x wk2; 1/wk x wk2

MSW Frequency:

PT Frequency: For Evaluation and Treatment. 2/wk x wk3, 1/wk x wk1 effective on 04/12/2026.

OT Frequency: For Evaluation and Treatment

HHA Frequency:

ST Frequency

Dietitian Frequency:

*****MD SIGNATURE COVERS ALL PAGES*****
ADMIT PATIENT TO ACTIVE HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

REASON FOR HOMEBOUND:
The patient is considered homebound because they have a symptomatic pathologic fracture of the right fibula with an expansile, infiltrative destructive lesion and soft-tissue extension on MRI, producing moderate tenderness and aching with weightbearing (pain 7/10) that limits safe ambulation and makes exiting the home without assistance unsafe. Objective findings supporting homebound status include: pathologic fracture of the right fibula on plain radiograph with bridging callus, an expansile destructive fibular lesion with soft-tissue extension on MRI concerning for sarcoma, a palpable mass with localized moderate tenderness on exam, and documented pain with weightbearing. These conditions create a risk of further fracture, acute pain exacerbation, and soft tissue compromise with ambulation; the patient is elderly (77 years) and has comorbid chronic lymphocytic leukemia and is taking antiplatelet therapy (clopidogrel), which increase peri-procedural risk and complicate safe transport and unsupervised activity outside the home. Leaving the home requires a considerable and taxing effort because any weightbearing provokes significant pain, there is structural instability of the fibula due to the pathologic fracture and destructive lesion, and there is no home assistance available to provide safe transfer or supervised mobility. Allowable, limited absences are restricted to medically necessary trips (preoperative evaluation, diagnostic imaging, and planned surgical care) only. Given the need for pre-operative evaluation and coordination, peri-procedural risk management, skilled wound and pain management planning, and close monitoring of a malignant/pathologic fracture, these objective limitations meet Medicare homebound criteria and justify the medical necessity for skilled home health services.
FREQUENCY AND DURATION:
HH SN frequency: 2 times a week for 2 weeks, 1 time a week for 2 weeks for skilled observation and assessment, medication reconciliation, pain assessment, wound monitoring and management, infection control measures, diet, home safety and disease process education.
HH PT frequency: 2 times a week for 3 weeks, 1 time a week for 1 week for evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training effective on 04/12/2026.
HH OT frequency: For evaluation and treatment focused on improving activities of daily living (ADLs), fine and gross motor skills, cognitive-perceptual function, adaptive techniques, and caregiver education.

GOALS:
-Patient will verbalize effective use of prescribed pain-management strategies (medication, positioning, cold/heat therapy) with pain reduced to ≤4/10 during weightbearing activities within 4 weeks.
-Patient will demonstrate understanding of wound monitoring and report no signs of infection or soft-tissue compromise at the fracture site within 4 weeks.
-Patient will correctly identify purpose, dosage, and schedule of all prescribed medications, including antiplatelet therapy, with 100% accuracy during medication reconciliation within 4 weeks.
-Patient will verbalize and demonstrate 3 safe transfer and fall-prevention strategies to reduce risk of further fracture or injury during mobility within 4 weeks.
-Patient will verbalize understanding of the pathologic fracture, malignant lesion, and peri-procedural risk factors (including impact of chronic lymphocytic leukemia and antiplatelet therapy) to support informed decision-making and compliance with care plan within 4 weeks.

Conference With:

- ✓ MD
- ✓ SN
- ✓ PT
- ✓ OT
- ☐ ST
- ☐ MSW
- ☐ HHA
- ☐ DON/DOPCS
- ✓ Clinical Coordinator
- ✓ Clinical Manager

Name:
Aimee Floresca, RN, CM

Regarding:
Start of care, plan of care, patient's status and frequencies.

Physician Contacted Re:
Start of care, plan of care, patient's status and frequencies.

Order Changes:

Plan for Next Visit:
Will continue to assess and evaluate patient understanding, functional status, and knowledge of self-care management on the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge (POC Element (§484.60 (2.xiii))):

Initiated education and training provided by the SN, as appropriate, regarding the care and services identified in the plan of care.

Discharge Plans

(POC Element):

When patient knows when to notify physician.
Discharge when reliable caregiver available to assist with patient's medical needs.
Discharge when goals met. Discharge when wound(s) healed.
Discharge when patient is independent in management of medical needs.
When goals are met.
Able to return to an independent level of care.
Able to understand medication regime and care related to diagnoses.
Discharge to caregiver.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Able to stay in residence with primary caregiver's assistance or community agency's support.
Discharge to care of physician.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge patient to self care.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

☑ **Patient/Family** ☑ **Care Manager** ☑ **Physician** ☐ **Other (specify):**

Discharge Plan:

☑ **To self-care when goals met** ☑ **To self care when max potential achieved** ☐ **To outpatient therapy with MD approval**

☐ **Others (Specify):**

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

(POC Element):

Edward Carriere is a 78-year-old male patient who has been referred for home health care services on March 27, 2026. He is currently under the care of attending physician Ronald Hillock. The assessment for home health care was completed by registered nurse Bright Dizon during the start of care visit on April 8, 2026. Notably, Edward was not discharged from any inpatient facility prior to this referral, indicating that his need for home health services arises from his current health status rather than a recent hospitalization.

Edward is classified as homebound due to his medical condition. His homebound status meets the criteria as he is unable to leave his home without considerable effort due to illness and pain following recent surgery. This includes significant weakness and pain that impairs his mobility, as well as an unsteady gait that increases his risk of falls. He requires assistance to ambulate and experiences exhaustion when attempting to leave home, which has been further exacerbated by the challenges associated with his recovery.

The primary reason for Edward's need for home health services is his diagnosis of C40.21, which refers to a malignant neoplasm of the long bones in the right lower limb. Additionally, he has secondary diagnoses that include M84.469D, indicating a pathologic fracture of the tibia and fibula, chronic lymphocytic leukemia (C91.10), essential hypertension (I10), and hyperlipidemia (E78.5). These conditions collectively contribute to his overall health challenges and necessitate skilled nursing care.

A comprehensive head-to-toe assessment reveals several key findings. Cardiopulmonary assessment shows stable vital signs with a temperature of 97.8°F, pulse rate of 76 beats per minute, respiratory rate of 18 breaths per minute, and blood pressure of 115/68 mmHg. Gastrointestinal assessment indicates that while Edward is able to feed himself, he requires assistance with setup and cleanup. The integumentary system assessment shows no signs of pressure ulcers or skin breakdown, but he has a surgical wound that is not currently observable due to a non-removable dressing.

Psychosocially, Edward lives alone and has a non-agency caregiver providing assistance. His emotional state appears stable; he expresses feelings of mild depression but denies any suicidal or homicidal ideation. He reports feeling lonely or isolated rarely, indicating a relatively positive psychosocial environment despite his health challenges.

Vital signs are well within normal limits, and Edward's cognitive function is intact. He is oriented to time, able to follow commands, and shows no evidence of cognitive decline. Activities of Daily Living (ADLs) show that he can manage some tasks independently, such as dressing his upper and lower body if clothing is prepared for him. However, he requires assistance with bathing, toileting, and grooming.

Edward's functional ability is categorized as independent in self-care and indoor ambulation. However, he does require assistive devices for safe mobility, such as a cane, and needs human assistance for certain tasks. His mobility is limited due to post-surgical pain, and he experiences dyspnea when walking more than 20 feet or climbing stairs.

Socially, Edward's needs include access to assistance for transportation and activities outside the home, given his homebound status. The assessment also indicates a significant number of medications being taken, with five or more prescribed medications that require careful management to prevent potential adverse effects and interactions. His medication regimen includes high-risk drugs such as opioids and antiplatelet agents.

Edward's primary caregiver is a non-agency individual, but additional support systems are in place, including emergency preparedness planning for potential natural disasters. This includes a documented emergency contact list and plans for managing medical needs in emergencies.

In conclusion, the medical necessity for skilled home health services for Edward Carriere is clear. He requires ongoing skilled nursing care to manage his complex medical needs and to support his recovery from surgery. The recommended frequency of visits is at least three times a week to ensure adequate monitoring of his health status, medication management, and assistance with activities of daily living, thereby promoting his safety and well-being in the home environment.

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness. Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.
Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue. Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN

Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure. Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest. Advise the patient to get up slowly to offset the feeling of dizziness. Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.

Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate. Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Malignant neoplasm of long bones of right lower limb
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage pain effectively and improve comfort levels
Interventions: Assigned Staff: SN
Assess and monitor pain levels regularly to adjust medication as needed.
Progress Toward Goals:
No Progress Yet
Provide education on pain management techniques, including medication and non-pharmacological methods.
Progress Toward Goals:
No Progress Yet
Encourage participation in relaxation techniques (e.g., deep breathing exercises).
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance mobility and functional independence
Interventions: Assigned Staff: SN
Monitor the patient's progress and adjust the therapy program as necessary.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain nutritional status and prevent weight loss
Interventions: Assigned Staff: SN
Assess dietary needs and provide tailored nutritional counseling.
Progress Toward Goals:
No Progress Yet
Monitor weight and nutritional intake regularly to ensure adequacy.
Progress Toward Goals:
No Progress Yet

Problem: Pathological fracture of the tibia and fibula and subsequent care for fracture with routine healing
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Ensure ongoing assessment of fracture healing progress
Interventions: Assigned Staff: SN
Conduct regular nursing assessments to monitor the healing process.
Progress Toward Goals:
No Progress Yet
Facilitate communication with the physician regarding any concerns with healing.
Progress Toward Goals:
No Progress Yet
Provide the patient with written materials on healing and recovery expectations.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Support the patient’s transition to increased mobility
Interventions: Assigned Staff: SN
Provide psychosocial support to address any mental health concerns during recovery.
Progress Toward Goals:
No Progress Yet

Problem: Chronic lymphocytic leukemia of B-cell type not achieving remission
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage symptoms and improve quality of life
Interventions: Assigned Staff: SN
Provide education to the patient and family about the disease and treatment options
Progress Toward Goals:
No Progress Yet
Monitor vital signs and symptoms regularly to assess changes in health status
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance communication with the healthcare team and family
Interventions: Assigned Staff: SN
Schedule regular care coordination meetings to discuss the patient's progress and needs
Progress Toward Goals:
No Progress Yet
Provide assistance in understanding medication regimens and potential side effects
Progress Toward Goals:
No Progress Yet
Develop a clear communication plan that includes emergency contacts and care preferences
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Ensure adherence to treatment protocols and follow-up appointments
Interventions: Assigned Staff: SN
Implement a medication management system to ensure adherence to prescribed therapies
Progress Toward Goals:
No Progress Yet
Educate the family on the importance of reporting any side effects or complications
Progress Toward Goals:
No Progress Yet
Conduct regular assessments to evaluate treatment responses and adjust plans as necessary
Progress Toward Goals:
No Progress Yet

Problem: Essential (primary) hypertension
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Achieve and maintain blood pressure within the normal range (120/80 mmHg).
Interventions: Assigned Staff: SN
Educate the patient and family about the importance of monitoring blood pressure regularly and keeping a log.

Progress Toward Goals:
No Progress Yet
Develop a personalized medication management plan to ensure adherence to prescribed antihypertensive medications.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-up visits to assess blood pressure and adjust the care plan as necessary.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve the patient's understanding of lifestyle modifications to manage hypertension.
Interventions: Assigned Staff: SN
Conduct educational sessions with the patient and family about lifestyle changes, including diet, exercise, and stress management.
Progress Toward Goals:
No Progress Yet
Provide resources and materials on hypertension management for the patient and family.
Progress Toward Goals:
No Progress Yet
Facilitate communication between the patient and their healthcare provider to address any concerns regarding medication or lifestyle changes.
Progress Toward Goals:
No Progress Yet

Problem: Hyperlipidemia
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce LDL cholesterol levels to below 130 mg/dL.
Interventions: Assigned Staff: SN
Educate the patient and family about dietary changes to lower cholesterol.
Progress Toward Goals:
No Progress Yet
Monitor lipid panel levels regularly to assess progress.
Progress Toward Goals:
No Progress Yet
Provide medication counseling regarding statin use and adherence.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Increase HDL cholesterol levels to above 40 mg/dL for men and above 50 mg/dL for women.
Interventions: Assigned Staff: SN
Educate the patient and family on healthy fats to include in the diet (e.g., olive oil, nuts).
Progress Toward Goals:
No Progress Yet
Schedule regular follow-ups to review progress on HDL levels.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Educate the patient and family about the impact of hyperlipidemia on overall health.
Interventions: Assigned Staff: SN
Provide comprehensive education sessions on cardiovascular health.
Progress Toward Goals:
No Progress Yet
Develop a personalized care plan that includes family involvement.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the risk of falls in the home environment.
Interventions: Assigned Staff: SN
Educate the family on the importance of keeping pathways clear and using assistive devices as needed.
Progress Toward Goals:
No Progress Yet
Review and adjust medications that may contribute to dizziness or confusion.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance patient and family knowledge regarding fall prevention strategies.
Interventions: Assigned Staff: SN
Provide educational materials about fall risks and prevention strategies.
Progress Toward Goals:
No Progress Yet
Set up regular check-ins with family to monitor progress and address concerns.
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving measures.
Interventions: Assigned Staff: SN
Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
The patient's skin remains intact.
Interventions: Assigned Staff: SN
Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores. Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (04/08/2026):

Assigned Staff: Dizon, Bright RN